Where You Are: Getting to Know the Person Is Part of Step Two, and What Comes After It

All nine steps of the Family Care Roadmap, and where her routines and preferences fit.

With new caregivers about to start, it can feel as if everything about your wife has to be explained at once. It doesn't. There's an order to it, and you're near the beginning, at the part that's about who she is.

This page lays out the whole road, marks where you are, and says what follows.

Phase 1 · Care Assessment

Understand the person, the family, and the situation.

1 **Discover** Current Situation → Concerns & Challenges → Goals & Priorities
You finish with: Family Discovery Summary

2 **Assess** Health & Function → Safety & Support → Lifestyle & Preferences
You finish with: Personal Care Assessment Summary

You are here. This is the third topic of the Assess step: lifestyle and preferences. Health and function came first, then safety and support, and now the person herself. The family's part is to get her routines and preferences written down, confirmed with her where possible, kept apart from anything a clinician has instructed, and to know who on the new team will receive them.

3 **Define Needs** Needs & Risks → Care Requirements → Priorities & Recommendations
You finish with: Care Needs Profile and Immediate Action Roadmap

Phase 2 · Care Planning

Determine what good care should look like and prepare to put it in place.

4 **Explore Options** Care Setting → Provider/Facility Fit → Selection & Decision
You finish with: Care Options Comparison and Decision Record

5 **Build the Plan** Health & Personal Care → Daily Life & Quality of Life → Monitoring & Response
You finish with: Personal Care Proposal, Good Day Plan, and Monitoring Plan

6 **Prepare the Transition** Preparation → Move & Admission → Continuity of Care
You finish with: Transition Preparation Pack and Confirmed Handoff Log

Phase 3 · Care Management

Coordinate the plan, review what happens, and prepare the family to take the lead.

7 **Coordinate** People & Responsibilities → Communication & Handoffs → Care-Plan Alignment
You finish with: Care Team Plan and Care-Plan Alignment Record

8 **Monitor** Monitor → Identify & Escalate → Review & Adjust
You finish with: Care Review Dashboard and Concern/Change Logs

9 **Hand Off** Educate → Empower → Support

You finish with: Family Care Management Playbook and Handoff/Support Plan

What this step produces

A short written picture of your wife as a person: what she likes to be called, how her days start and end, what she likes to eat, what she enjoys and who matters to her, how to talk with her, and what helps when she's unsettled. It is her account and the family's, organized. It isn't an assessment and it isn't a care instruction.

Together with the two earlier topics of this step, her health and function and her safety and support, it goes into the Personal Care Assessment Summary.

What happens next

This topic closes the Assess step. The next step, Define Needs, brings the three parts together and states what care has to cover. After that the roadmap moves on to exploring options and building the plan, where daily life and quality of life have a place of their own. What you write down now is what that part starts from.

The page isn't final. For nursing homes, Medicare's guide for residents says preferences and goals may change during a stay, so the home should have ongoing discussions with the resident about updating the care plan, and the National Consumer Voice says the team that develops the plan should have input from the resident and family members. Both of those are about nursing homes. Wherever she'll be, ask how her preferences get updated. [1] [2]

Who is involved, by role

- Your wife: her own account of her day, and her say over what is shared.
- You and the family: asking, writing down, marking which lines are hers and which are yours, and getting the page to the new team.
- Her clinicians (doctor, nurse, dietitian): allergies, prescribed diets and any other instruction, and what to do with a note you can't place.
- The new care team: receiving the page and telling you who has it. In a nursing home, the National Consumer Voice says, an assessment typically involves the resident, nursing staff, social services staff, dietary staff, activities staff and family members.
- And your care consultant: records her morning routine, meal times, food preferences and evening routine; keeps preferences, restrictions, allergies and prescribed instructions distinct instead of treating them as interchangeable; and talks about the activities she enjoys and the help she'd need to take part.

What this page doesn't promise

How long any step takes, who will be available, whether a particular routine can be kept in the new setting, or how things turn out. Every family takes the steps at its own pace.

Ready for the next step?

You know where this piece sits and what follows it. The care interview is how the Assess step gets finished: it asks about her preferences alongside her health, her safety and the support around her, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The nine steps are the Family Care Roadmap's own. The points about updating a care plan and about who takes part in an assessment come from the two sources below, both written about nursing homes.

1. Centers for Medicare & Medicaid Services, Your guide to living in a nursing home (CMS Product No. 12217)
2. National Consumer Voice for Quality Long-Term Care, Family Involvement in Nursing Homes (fact sheet)
